World Health Organization

ACCESS TO MEDICINES AND HEALTH PRODUCTS DIVISION

WHO guideline on balanced national controlled medicines policies to ensure medical access and safety
Rapid communication

Background

Access to medicines is a key component for attainment of universal health coverage, which is central to achievement of the health-related Sustainable Development Goals. Controlled medicines include opioids, benzodiazepines, barbiturates, amphetamines and others with identified or emergent clinical indications. WHO recognizes that these medicines are necessary for pre- and post-operative care, for sedation, for the management of both acute and chronic pain, for palliative care, as anticonvulsants (anti-epileptics), for the management of anxiety disorders, and for the management of substance use disorders, including as opioid agonist therapy.

WHO recommends that essential medicines, including those that are controlled, be available to all patients at all times at a price that the individual and the community can afford. In line with their obligations under the United Nation's drug treaties, governments and health systems must ensure that people who need controlled medicines for medical and scientific purposes can access them and also ensure that these medicines are used safely and appropriately. Policies should seek to maximize access to essential and beneficial controlled medicines for all people who need them, while effectively restricting non-medical use, which poses serious risks to safety and has harmful consequences for individuals and societies.[1] This guideline is aligned with the WHO road map for access to medicines, vaccines and other health products.[2] It provides evidence-informed recommendations on pharmaceuticals with an identified or emergent clinical application the active principles of which are listed in international drug control conventions and the manufacture, possession and use of which are regulated by national law.

Purpose of the guideline

The purpose of this guideline is to assist WHO Member States and their partners in developing and implementing balanced national controlled medicines policies to ensure their accessibility, availability and affordability for medical and scientific uses and to minimize the risk of harm arising from non-medical use. The guideline addresses policies for groups (of all ages, from neonates to older people) affected by conditions in which the use of controlled medicines is deemed to be medically appropriate according to evidence-based clinical practice guidelines.

Scope of the guideline

Populations

The recommendations in this guideline are relevant to policies addressed to groups (of all ages) affected by conditions for which the use of internationally or nationally controlled medicines is deemed to be medically appropriate according to evidence-based guidelines.

The guideline includes consideration of the necessity of adapting policies to meet the specific needs for access and safety of patients in various demographic groups, including neonates, children, adolescents, young people, adults and older people. The guideline also includes the needs of vulnerable populations.

---

[1] Resolution WHA67.22. Access to essential medicines. In: Sixty-seventh World Health Assembly, Geneva, 19–24 May 2014. Resolutions and decisions, annexes. Geneva: World Health Organization; 2014:2 https://apps.who.int/gb/ebwha/pdf_files/wha67-rec1/a67_2014_rec1-en.pdf

[2] Roadmap for access to medicines, vaccines and health product 2019–2023: comprehensive support for access to medicines, vaccines and other health products. Geneva: World Health Organization; 2019. https://iris.who.int/handle/10665/330145

Clinical contexts

Controlled medicines are most likely to be used in the following contexts:

- anaesthesia and procedural, pre- and post-surgical care;
- disease conditions associated with acute pain and chronic and non-chronic cancer pain;
- palliative and hospice care;
- management of mental health disorders;
- management of substance use disorders;
- management of neurological conditions, including seizures and severe spasticity;
- other relevant conditions, such as sickle-cell disease;
- clinical research on medical applications of controlled substances; and
- humanitarian emergencies and crises.

Some clinical conditions, notably acute and chronic pain, trauma surgery and acute mental health problems, are particularly likely to arise during humanitarian emergencies, including those due to climatic or geological disaster, political or ethnic conflict or serious infectious disease epidemics. These conditions and circumstances were considered during guideline development.

Types of controlled medicines

The recommendations in this guideline are intended to cover all types of controlled medicines with authorized medical or scientific purposes. They include opioids, benzodiazepines, barbiturates, dissociative anaesthetics, cannabinoids, hallucinogens and amphetamine-type stimulants.

Policy areas considered to be out of the scope of this guideline

This guideline addresses policies rather than clinical practice and therefore does not provide advice on clinical use of controlled medicines.

Guiding principles

Several key principles underpin the guideline and apply to all national policies pertaining to controlled medicine.

1. All people have the right to enjoyment of the highest attainable standard of health, a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity.
2. Controlled medicines are crucial for managing many health conditions and treating illness. Access to essential controlled medicines is a component of the rights to both health and life.
3. National policies pertaining to controlled medicines should be balanced to enable safe and appropriate use, ensuring access for medical and scientific need (that is uninterrupted, sustainable and continuous) while avoiding harmful consequences for individuals and societies.
4. All national policies should be tailored to the needs and requirements of the social context and resources of the population, while recognizing individuals' right to the highest attainable standard of health.
5. Member States and health-care providers should ensure that patients, their families and their caregivers know their rights to self-determination, non-discrimination, accessible and appropriate health services and confidentiality.

6. Governments should invite patients, advocacy groups, health professionals, academia, professional societies, civil society and other affected groups to participate in formulation of health policy.

Existing WHO guidance (domains 1–3)

Existing WHO guidance and recommendations are included in chapters 4,5, and 6 of this guideline to provide support for the development of a national policy on controlled medicines, pricing and financing of controlled medicines and related health care services, and medicines selection.

Chapter 4. Development of a national controlled medicines policy (domain 1)
WHO guidance (2001): WHO recommends that all countries develop and implement a national medicines policy, that they regularly monitor its implementation and update it to ensure that its goals remain in line with national medical needs and social priorities, as well as the most recent international norms.[3]
Chapter 5. Pricing and financing of controlled medicines and related health-care services (domain 2)
WHO guidance (2020): WHO recommends that countries enable early market entry of generic and biosimilar medicines through legislative and administrative measures, with a view to encouraging early submission of regulatory applications, allowing for prompt, effective review and ensuring that these products are safe, efficacious and quality assured.[4]
WHO guidance (2020): WHO recommends that countries use multiple pricing policies to achieve low prices for generic and biosimilar medicines that are based on the cost of production.
WHO guidance (2020): WHO recommends maximization of the uptake of generic and biosimilar medicines.
Chapter 6. Medicines selection (domain 3)
WHO guidance (2001, 2019): WHO recommends that selection of all medicines be based on a transparent, rigorous assessment of the latest available scientific studies.[5] In the case of controlled medicines that are not on the Essential Medicines List, the review of evidence should explicitly include studies in which the risks of inappropriate and unsafe use are quantified in the national context, as well as the relative therapeutic value of different formulations of controlled medicines. When such evidence does not exist, WHO encourages its generation.[6]
WHO guidance (2019) WHO's guidelines on medicine pricing recommend use of health technology assessment or equivalent tools or approaches in selecting medicines for public coverage, so that the

[3] How to develop and implement a national drug policy, second edition. Geneva: World Health Organization; 2001. https://iris.who.int/handle/10665/42423

[4] WHO guideline on country pharmaceutical pricing policies, second edition. Geneva: World Health Organization; 2020. https://iris.who.int/handle/10665/335692

[5] How to develop and implement a national drug policy, second edition. Geneva: World Health Organization; 2001. https://iris.who.int/handle/10665/42423

[6] Roadmap for access to medicines, vaccines and health product 2019–2023: comprehensive support for access to medicines, vaccines and other health products. Geneva: World Health Organization; 2019. https://iris.who.int/handle/10665/330145

process is transparent, the assumptions are explicit and the perspectives of patients and buyers are taken into account.[7]

New recommendations and good practice statements on ensuring safe, balanced access to controlled medicines (2025)

New recommendations and good practice statements published by WHO in 2025 are included in chapters 7, 8, 9, and 10 of the guideline. The recommendations are based on the best available scientific evidence and include ratings of the certainty of evidence.[8] The recommendations were formulated with methods that meet the highest international standards for guideline development.[9] Rapid systematic reviews were conducted to synthesize the evidence, and all components of the WHO INTEGRATE Evidence-to-decision framework (balance of health benefits and harms, human rights and sociocultural acceptability, health equity, equality and non-discrimination, societal implications, financial and economic considerations, feasibility and health system considerations, as well as the quality of the evidence)[10] were considered in recommendation development.

While the recommendations were based on the best available evidence, good practice statements were also formulated to represent good practice for policies related to the control of medicines. The good practice statements were supported by indirect evidence and do not include a rating of the certainty of evidence.

Chapter 7. Procurement and supply chain management (domain 4)
7.1 Quantification of controlled medicines
Strong recommendation: Governments should ensure that reporting of current consumption and the potential need for controlled medicines for medical and scientific purposes, including ongoing adjusted estimates where necessary, are: • accurate, timely and actively monitored and • based on need that is estimated from the best available epidemiological data (including morbidity and mortality data), consumption data, clinical guidelines, service capacity and other relevant information. *Very low certainty evidence*
7.1 Quantification of controlled medicines

[7] WHO guideline on country pharmaceutical pricing policies, second edition. Geneva: World Health Organization; 2020. https://iris.who.int/handle/10665/335692

[8] Dewidar O, Lotfi T, Langendam MW, et al. Good or best practice statements: proposal for the operationalisation and implementation of GRADE guidance. *BMJ Evid Based Med.* 2023;28(3):189-196.

[9] World Health Organization. WHO handbook for guideline development, second edition. Geneva: World Organization; 2014. https://iris.who.int/handle/10665/145714

[10] Rehfuess EA, Stratil JM, Scheel IB, Portela A, Norris S, Baltussen R. The WHO-INTEGRATE evidence to decision framework version 1.0: integrating WHO norms and values and a complexity perspective. *BMJ Glob Health.* 2019;25(4):e000844.

Good practice statement: Governments should monitor the availability and affordability of controlled medicines and update estimates of need to ensure adequate supplies on an ongoing basis. If the national supply, demand or other area of availability changes significantly, updated quantitative estimates should be made and communicated to the International Narcotics Control Board in a timely manner to allow corresponding changes to manufacture, importation and distribution.
Good practice statement: Governments should develop, implement and monitor their procurement policy for controlled medicines to ensure a sufficient, high-quality, efficacious, safe and cost-effective supply for medical and scientific needs, in compliance with rules and regulations for public sector procurement of health products.
Good practice statement: Governments should develop, implement and monitor good procurement policy to identify continually assessed and evaluated sources and achieve the best sustainable prices for quality-assured controlled medicines for medical and scientific needs.
7.3 Supply chain management systems, tools and mechanisms
Strong recommendation: Governments should use simple, appropriate technology and tools to: • improve the traceability, efficiency and integrity of inventory management for controlled medicines; • prevent waste and stock-outs; • implement protocols for prevention of diversion; and • reduce the administrative burden for front-line staff handling controlled medicines. *Very low certainty evidence*
Good practice statement: Governments should have supply chain and distribution plans for controlled medicines that ensure full geographical coverage, prevent waste or shortages and avoid inequity in access.
Good practice statement: Governments should ensure that local and regional productions hubs and supply chains are supported by adequate technologies, infrastructure, financial and human resources.
7.4 Local production of controlled medicines
Good practice statement: Governments should systematically collect and analyse information on the potential health, financial and social benefits as well as the risks and harms of producing quality-assured controlled medicines within their countries to meet the health-care needs of their people.

Chapter 8. Medicines regulation and control (domain 5)
8.1 Medicine product safety
Good practice statement: Governments should ensure that controlled medicines are available in formulations that are acceptable, affordable and accessible for those with clinical need.

Good practice statement: Governments should ensure that packaging of controlled medicines prevents accidental use by children and vulnerable adults. The additional cost of ensuring such safety features should not result in reduced access for patients with clinical need.
Good practice statement: Governments that are considering adoption of tamper-resistant formulations or packaging should weigh their potential safety benefits against their higher cost and the risk that they may limit access and/or increase harm.
8.2 Possession and use of controlled medicines
Good practice statement: Governments should collaborate with health authorities, care providers, professional health organizations and patient advocacy groups to review laws and regulations regarding the possession and use of controlled medications. They should revise any laws that hinder access to these medicines for individuals with legitimate clinical needs.
Good practice statement: Governments, medicines regulatory agencies, health-care professional bodies and societies should ensure that permission to possess or handle controlled medicines is extended to all health professionals whose practice entails treating patients with a clinical need for controlled medicines.
Good practice statement: Governments should ensure that patients have adequate legal protection relating to the possession of prescribed controlled medicines for clinical need.
8.3 Drug scheduling
Good practice statement: Governments should ensure that changes to the scheduling of controlled medicines are based on robust scientific evidence relevant to the context of use, to achieve balance between ensuring access and preventing public health harm.
Good practice statement: Discussions of drug scheduling should include input from health authorities, associations of health professionals, patients, families and all relevant stakeholders. When reviewing drug scheduling, optimizing health outcomes should be the priority, balancing access for clinical need with preventing harm.
Good practice statement: Governments and relevant authorities should ensure that medicine scheduling does not impede access to controlled substances for use in ethically approved clinical research.
8.4 Regulatory procedures for import and export
Good practice statement: Governments and relevant authorities should review their requirements and procedures governing trade of controlled medicines to ensure that they are proportionate, do not obstruct the flow of medicines necessary to treat people with clinical need and competently minimize diversion.
Good practice statement: Governments should encourage universal use of electronic authorizations and reporting in implementing trade regulations for controlled medicines.
Good practice statement: Governments and relevant authorities should ensure that controlled medicines can be exported or imported rapidly for use by humanitarian response organizations that are duly authorized by relevant national authorities.

Chapter 9. Prescribing, dispensing, and administration (domain 6)
9.1 Clinical practice guidelines
Good practice statement: Governments and relevant authorities should ensure that clinical guidelines for controlled medicines are: • developed or revised by independent experts according to a scientific process based on the best available evidence; • designed to optimize access to controlled medicines that are safe, effective and appropriate for all patients with clinical need; • are non-discriminatory and address the needs of specific and vulnerable populations; and • are designed to protect the population from harm.
9.2 Regulations or policies governing prescription, dispensing and administration
Strong recommendation: Governments should develop and implement policies to ensure that prescription, dispensing and administration of opioid agonist treatment are available for people with opioid dependence in all settings in which there is clinical need and should ensure continued access throughout transitions of care. The settings include communities, prisons and other closed settings. *Low certainty of evidence*
Good practice statement: In settings where there is high prevalence of non-medical use of certain controlled medicines, with associated harm, policies for prescription, dispensing and administration of controlled medicines should be implemented to limit diversion to non-medical use without reducing access for those with clinical need or for the purpose of scientific research.
Good practice statement: Governments and relevant authorities should ensure that regulations and guidelines on prescription, dispensing and administration of controlled medicines should be formulated to optimize safe, effective, equitable, convenient access for those with clinical need, while maintaining proportionate safeguards against potential harm.
Good practice statement: Governments and relevant authorities should ensure that regulations enable health professionals to prescribe, dispense and administer controlled medicines without undue barriers, allowing them to work to the full scope of their practice, to ensure that controlled medicines are accessible to patients with clinical need.
9.3 Prescription monitoring programmes and pharmacovigilance systems
Good practice statement: Governments should ensure robust nationwide systems that allow monitoring of prescriptions for controlled medicines, with specific attention to the protection of patient privacy, to optimize access to and safe use of controlled medicines. Where feasible and affordable, electronic systems should be prioritized. The absence of these systems should not be a barrier to access to clinically necessary controlled medicines.

Good practice statement: Governments should ensure that systems for monitoring prescriptions of controlled medicines should not be used to expose caregivers, patients or prescribers to unwarranted scrutiny in the delivery or receipt of clinically indicated health care.
Good practice statement: Governments should ensure active monitoring of the safety of controlled medicines, including new formulations, through a robust pharmacovigilance programme.
9.4 Pharmaceutical industry relations
Good practice statement: Governments should implement national regulations to ban misleading or unethical marketing of controlled medicines to patients, health-care providers and other stakeholders involved in medicine purchase or supply.
Good practice statement: Governments, professional societies and international organizations should implement robust, transparent policies to prevent and manage conflicts of interest in the training, education and promotion of products to health professionals and in the development of clinical guidelines. This includes both direct and indirect commercial influence via patient groups or other stakeholders.
Good practice statement: Governments should implement policies to address undue influence, including preventing and managing conflicts of interest of legislators, regulators and other government officials who may formulate or vote on legislation or rules for controlled medicines.

Chapter 10. Education, knowledge and attitudes (domain 7)
10.1 Training for health-care professionals
Strong recommendation: Governments, academic institutions and other responsible bodies should promote comprehensive training in adequate access and safe use of controlled medicines, according to clinical guidelines, in core curricula and continuing professional education programmes of relevant health-care disciplines. *Very low certainty evidence*
10.2 Patient and public education
Strong recommendation: Governments should ensure, through appropriate authorities and institutions, the delivery of balanced, accurate information about controlled medicines to patients, families, caregivers and the public. Information should be provided about the potential benefits and risks of therapeutic use and also of the potentially serious risks associated with non-medical use. *Low certainty evidence*

Monitoring, evaluation and research

In several policy areas addressed by this guideline, there is no published scientific literature. This gap may have arisen for several reasons, including the inherent difficulty in designing studies to test national policies, especially when those policies are shaped by international law, limiting experimental opportunities. Additionally, it is challenging to assess and quantify the extent of the unmet clinical need for conditions treated with controlled medicines. Non-medical use of controlled medicines is

often illegal, making it difficult to track and compare the desired end-point of policies designed to decrease both unmet need and non-medical use.

In some areas, research on the outcomes of policies intended to increase access to controlled medicines did not include the impact of such policies on safety-related outcomes. Conversely, most studies of policy interventions for reducing harm from controlled medicines did not include evaluation of their impact on access outcomes.

Studies in low- and middle-income countries were under-represented in the academic literature. This may be due partly to structural biases related to language and research funding but also coincides with more limited access to controlled medicines in those regions. Historically, the problems of overuse and oversupply of controlled medicines have been largely limited to high-income countries, and it is not appropriate to generalize these findings to countries with less access.

Areas for further monitoring, evaluation and research highlighted in the guideline include the following.

- The impact of interventions for quantification, procurement, supply chain and local production of other controlled medicines for alternative indications should be further studied.
- Research gaps include management of waste of controlled medicines, donations, strategies to address substandard and falsified medicines and implementation approaches for local production.
- Little evidence is available on potential strategies to increase access through regional hubs for the production of controlled medicines according to pooled need.
- Additional research is required to understand the impacts of up-scheduling and down-scheduling access to controlled medicines and the potential safety outcomes.
- The feasibility and acceptability of using electronic authorizations for controlled medicines should be investigated internationally, including the clinical and societal outcomes.
- Providers face moral and cultural concerns and health system barriers, which vary by context. Research should be conducted on the effects of policies and educational interventions in various therapeutic and cultural contexts and on their impacts in low-access countries in the long-term, as well as the potential harms and the impact of public education materials on attitudes and behaviour.
- Better understanding is necessary of the influence of social media on safe use of controlled medicines.
- Research should be conducted on the effectiveness of public education campaigns, and impactful platforms should be identified.

Next steps

The 2025 WHO guideline on balanced national controlled medicines policies to ensure medical access and safety updates and replaces the WHO document "Ensuring balance in national policies on controlled substances: Guidance for availability and accessibility of controlled medicines" (2011).[11]

The full WHO guideline on balanced national controlled medicines policies to ensure medical access and safety will be released in 2025.

Acknowledgements

WHO gratefully acknowledges the work of the members of the Guideline Development Group, the reviewers of the evidence and the members of the external review group.

© World Health Organization 2025. Some rights reserved. This work is available under the CC BY-NC-SA 3.0 IGO licence.

Suggested citation. WHO guideline on balanced national controlled medicines policies to ensure medical access and safety: rapid communication. Geneva: World Health Organization; 2025. https://doi.org/10.2471/B09419

[11] Ensuring balance in national policies on controlled substances: guidance for availability and accessibility of controlled medicines. Geneva: World Health Organization; 2011. https://iris.who.int/handle/10665/44519